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CLINICAL GUIDELINE

This guideline is intended to provide guidance to applicants wishing to submit applications for the registration of Human Medicines containing specified substances. In addition to this guideline, SAHPRA reserves the right to request any additional information to establish the quality, safety, and efficacy of a medicine in keeping with the knowledge current at the time of evaluation. SAHPRA is committed to ensure that all registered medicines will be of the required quality, safety, and efficacy.

Document History

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3	- Content structured on the new SAHPRA Guideline Template - Old guideline number 2.09 changed to SAHPGL-CEM-01	August 2022
4	The following Sections have been updated, this includes: - Glossary table to cover all abbreviations used in this guideline; - Section 1 (Introduction) includes reading with ICH guidelines; - Section 3, added to indicate the Types of applications; - Section 4 (Clinical review type details) previously defined as section 3, recognition review pathway deleted in line with current clinical review pathways; - Section 5 (Pre-clinical and clinical studies) previously defined as section 4, part (d) has minor editorial change; - Section 6 (Document/ Data Requirements for New Registration) previously defined as section 5; reference to eCTD modules updated and minor editorial change now included; - Section 7 (References) previously defined as section 6 now revised to include relevant guidelines;	June 2026

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Glossary

Abbreviation/ Term	Meaning
Act	Medicines and Related Substances Act, 1965 (Act 101 of 1965 as amended)
Amendments	Used interchangeably with the term ‘variations’
API	Active Pharmaceutical Ingredient
Authority	Refers to SAHPRA
CHMP	Committee for Medicinal Products for Human Use
EA	Extension Application
EMA	European Medicines Agency
EU	European Union
FAAF	Formulation As Applied For
FPP	Finished Pharmaceutical Product
Generic	This term is used interchangeably with “interchangeable multisource medicine” which means medicines that contain the same active substances which are identical in strength or concentration, dosage form and route of administration and meet the same or comparable standards, which comply with the requirements for therapeutic equivalence as prescribed.
GCP	Good Clinical Practice
ICH	The International Council on Harmonisation of Technical Requirements for Registration of Pharmaceuticals for Human Use
MAH: Equivalent to HCR	Market Authorisation Holder: Holder of the Certificate of Registration
MHLW	Ministry of Health, Labour and Welfare (Japan)
MHRA	Medicines and Healthcare Products Regulatory Agency (United Kingdom)
NCE	New Chemical Entity
Q & BE	Quality and Bioequivalence
PBRER	Periodic Benefit-Risk Evaluation Report
Pharmaceutical alternatives	Medicines that contain the same active moiety but differ either in chemical form of that moiety or in the dosage form or strength; and after administration in the same molar dose, their effects with respect to both efficacy and safety are essentially the same.

Pharmaceutical equivalent	Medicines that contain the same amount of active substances in the same dosage form, meet the same or comparable standards and are intended to be administered by the same route.
PI	Professional Information
PIL	Patient Information Leaflet
PSUR	Periodic Safety Update Report
RMP	Risk Management Plan
RRA	Recognised Regulatory Authorities – a term used to refer to the list of regulatory authorities with which SAHPRA aligns itself
SAHPRA	South African Health Products Regulatory Authority
SAP	Statistical Analysis Plan
SmPC	Summary of Product characteristics: Equivalent to PI
The Authority	Relevant regulatory authority, in this case SAHPRA
TGA	Therapeutic Goods Administration (Australia)
Unregistered medicine	Is a medicine that were on the South African market before registration became mandatory with the promulgation of The Medicines and Related Substances Control Act, 1965 (Act 101 of 1965). These medicines were never formally evaluated by the SAHPRA or former MCC for safety, efficacy and quality.
US FDA	United States Food and Drug Administration
WHO CRP	World Health Organisation Collaborative Registration Procedure

1. INTRODUCTION

The science and practice of drug development and clinical evaluation have evolved significantly, and this evolution has implications for the amount and type of data needed to support effectiveness in certain cases. Guidelines are constantly evolving because of scientific developments and harmonisation of the requirements of the major international regulatory authorities. The South African Health Products Regulatory Authority (SAHPRA) endeavours to keep abreast of such developments and keep its application requirements and evaluation policies in line with international best practice. As an Assembly to The International Council on Harmonisation of Technical Requirements for Registration of Pharmaceuticals for Human Use (ICH), South Africa uses and applies information provided in the ICH guidelines for safety, efficacy, and quality requirements. This guideline must be read in conjunction with relevant ICH guidelines, including but not limited to E3 (Clinical Study Reports), E6 (Good Clinical Practice), E8 (General Considerations) and M4 (CTD Structure). This guideline adopts and refers to the Safety and Efficacy guidelines from ICH in their current form and/ or with amendments to align with local requirements and legislation.

The concepts of safety and therapeutic efficacy can only be examined in relation to each other and have only relative significance depending on the scientific knowledge and the use for which the medicine is intended. The particulars and documents which must accompany an application for registration of a medicine demonstrate that potential risks are outweighed by the therapeutic efficacy of the medicine.

Evaluation of safety and efficacy data also includes the evaluation and approval of the professional information (PI) and patient information leaflet (PIL) for registration purposes. It is the applicant's responsibility to maintain the life cycle of PI and PIL post registration.

SAHPRA in terms of the general regulations outlines in the Medicines and Related Substances Act, 1965 (Act 101 of 1965 as amended) requires that a PI and PIL to be made available for each medicine as stipulated in Regulation 11 and 12, respectively. The Authority requires that an application for the registration of a medicine be accompanied by a PI and PIL as prescribed by the Act. For the format and content requirements of the PI and PIL, refer to the current SAHPRA PI and PIL guidelines (SAHPGL-CEM-02).

This guideline outlines SAHPRA's evaluation pathways for Clinical and the associated documentation/ data requirements and provides guidance on Non-clinical and Clinical studies. Throughout the Common Technical Document, the display of information should be unambiguous and transparent, in order to facilitate the review of the basic data and to help a reviewer become quickly oriented to the application contents.

NOTE: An unregistered medicine PI cannot be used as reference source for a multisource/ generic PI. For

further explanation refer to the PI/ PIL guidelines (Guideline for Professional Information for Human Medicines (Categories A and D) and Guideline for Patient Information Leaflet for Human Medicines (Categories A and D).

1.1 Purpose

This document provides guidance on the review pathways for clinical evaluations and the required documentation/ data for the respective review pathways for the purpose of registration of Human Medicines (categories A and D) and the amendments thereof.

1.2 Scope

The guideline is applicable to categories A and D Human Medicines, as defined in the Medicines and Related Substances Act (Act 101 of 1965). In addition to this guideline, SAHPRA reserves the right to request any additional information to establish the safety, quality, and efficacy of a medicine in keeping with the knowledge current at the time of evaluation. SAHPRA is committed to ensure that all registered medicines will be of the required quality, safety, and efficacy.

2. LEGAL PROVISION

This guideline is based on the requirements of General Regulations 2, 10, 11, 12 and 15 issued in terms of the Medicines and Related Substances Act, (Act 101 of 1965), as amended (Government Notice No. 859, Government Gazette No. 41064, 25 August 2017).

3. TYPES OF APPLICATIONS

3.1 New chemical entity (NCE)

Applications (including clones) that include non-clinical and clinical information in support of the efficacy and safety of the formulation/ dosage form, indication(s) and dosage regimen.

3.2 Multisource/ generic applications (including replicas) and extension applications

That include (where applicable):

- clinical information in support of efficacy and safety of the formulation/ dosage form, or indication(s) or dosage regimen.
- comparative bioavailability/ bioequivalence studies as proof of efficacy.
- comparative dissolution studies as proof of efficacy.
- any other comparative studies as proof of efficacy.

- others, not mentioned above e.g., liquids/ solutions.

3.3 Multisource/ generic applications (pharmaceutical equivalence)

A multisource/ generic medicine is described as medicine which has:

- the same qualitative and quantitative composition in active substances in the same dosage form as the reference product registered by SAHPRA,
- the same pharmaceutical form and are intended to be administered by the same route as the reference product registered by SAHPRA,
- and whose bioequivalence with the registered product has been demonstrated by appropriate bioavailability studies (where applicable).

The different salts, esters, ethers, isomers, mixtures of isomers, complexes or derivatives of an active substance must be the same active substance, unless they differ significantly in properties regarding safety and/ or efficacy. In such cases, additional information providing proof of the safety and/ or efficacy of the various salts, esters, ethers, isomers or mixtures thereof or derivatives of an authorised active substance must be supplied by the applicant.

3.4 Multisource/ generic application (pharmaceutical alternative) and Extensions

Products are pharmaceutical alternative(s) if they contain the same molar amount of the same active pharmaceutical moiety(s) but differ in dosage form (e.g. tablets versus capsules), and/ or chemical form (e.g. different salts, different esters). Pharmaceutical alternatives deliver the same active moiety by the same route of administration but are otherwise not pharmaceutically equivalent. They may or may not be bioequivalent or therapeutically equivalent to the comparator product.

Therapeutic equivalence is determined from comparative bioavailability, pharmacodynamic, clinical or in vitro studies which meets the requirements and acceptable criteria for bioequivalence as determined by the Authority.

These types of applications depend partly on the results of tests on reference medicine and partly on new data from clinical studies.

3.4.1 Types of Extension applications:

Applications that may fall in these categories are (refer to the SAHPRA Guideline for submission of Extension applications):

- Change in active substance.
- Change in bioavailability.
- Change in pharmacokinetics (e.g., change in rate of release).
- change or addition of a new strength/ potency.
- change or addition of a new route of administration.

Pharmaceutical alternatives and Extension applications are regarded as generics with clinical data. Clinical data should be submitted to support the proposed application and the relevant ICH guidelines can be utilised for guidance.

4. CLINICAL REVIEW TYPE

A clinical application will follow one of the following review types, namely:

- (a) Full Review
- (b) Abridged Review
- (c) Verified Review

Review types (b) and (c) represent reliance pathways, which SAHPRA will be implementing to reduce evaluation times. To qualify for a reliance pathway, an application must have received prior approval from RRA. Reliance pathways are discussed in detail in the latest SAHPRA reliance guideline (SAHPGL-BAU-01).

Full review is defined as a comprehensive/ thorough review of all aspects of the dossier, based primarily on the evaluation of data (and summaries thereof) submitted by the applicant. This is the default evaluation pathway for new registrations and variations not previously approved by SAHPRA or RRA, or where reliance documentation provided to SAHPRA is deemed to be insufficient.

The final evaluation pathway decision for an application is at the discretion of SAHPRA and will depend on the availability and quality of reliance documentation submitted. SAHPRA will share screening queries with applicants regarding insufficient reliance documentation to ensure that as many applications as possible qualify for abridged and verified reviews. Applications will default to a Full Review in the absence of a suitable reliance pathway.

NOTE: That an application for an API that has not yet been registered by SAHPRA will be considered as a New Chemical Entity (NCE) in South Africa, regardless of whether the molecule has already been registered by other regulatory authorities.

4.1 FULL REVIEW

A full review involves a thorough review of all aspects of the dossier, particularly the pre-clinical and clinical data submitted under CTD Modules 4 and 5 (and summarised in Module 2). All NCE biological, and biosimilar medicine applications, generic applications with clinical data, Type II variations and extension applications (EAs) that lack adequate reliance documentation or do not have prior approval from an RRA will undergo/ be subjected to a full review pathway.

A full review is indicated specifically for the following types of applications:

4.1.1 Monocomponent medicines

- For a monocomponent NCE (new chemical entity) not registered by SAHPRA or an RRA,
- For a monocomponent multisource medicine/ generic/ API not registered by SAHPRA or an RRA, and where clinical data generated with the multisource/ generic has been supplied in support of the application, and
- Biological medicine not registered by SAHPRA or an RRA.

4.1.2 Multicomponent medicines

- For a multicomponent fixed dose combination of two or more chemical entities, where the combination is not registered by SAHPRA or by an RRA.

4.1.3 Type II variations

- For Type II variations where the amendment applied for has not been approved by an RRA or when the amendments are made to therapeutic indication and posology.

4.1.4 Extension applications

- For EAs which have not yet been approved by SAHPRA or by an RRA for a given molecule.

4.2 ABRIDGED REVIEWS

The Abridged Review is initiated to limit the evaluation time of medicines that are registered by RRAs.

All NCE and biological applications, generic applications with clinical data, Type II variations and EAs that

have prior approval from a RRA will be considered for an abridged review. In addition, all applications for biosimilar medicines will be considered for an abridged review.

For the requirements for an application to be considered for an abridged review refer to the SAHPRA reliance guidelines (SAHPGL-BAU-01).

4.3 VERIFIED REVIEW

The verified review is initiated to limit the evaluation time of multisource/ generic applications for APIs already registered by SAHPRA.

The verified review is effectively a comparison of an applicant's proposed PI against an approved up-to-date reference PI (from a Clinical safety perspective). The primary reference for a verified review is the latest-approved PI of the associated local innovator product.

The most recently approved Innovator SmPC/ PI from a recognised regulatory authority (RRA) (for multisource/ generic applications: as a supplement for safety aspects only in instances where the local IPI is materially outdated i.e. 5 years or older) refer to the Guideline on Professional Information for Human Use (see SAHPGL-CEM-02 for which sections require complete localisation to the SA innovator product).

All Type IB variations, and multisource/ generic applications (without clinical data) for APIs already registered by SAHPRA will be considered for a verified review.

5. PRE-CLINICAL AND CLINICAL STUDIES

5.1 PRE-CLINICAL STUDIES (CTD MODULES 2.4, 2.6 & 4)

The details of results from tests shall depend on the state of scientific knowledge at the time when the application is lodged. Any interim and final results of ongoing studies should be submitted as soon as this data become available. A new route of administration, or an increased daily dose, or different inactive pharmaceutical ingredients, may result in the need for additional pharmaco-toxicological data. Details (published or unpublished) of the results of any trials or experiments carried out in a man or in the animal target species, or carried out in other animals, that establish and confirm the safety of the medicine, with reference to the Posology and method of administration for use, should be included.

5.1.1 Pharmacology

The Pharmacology of the medicine should be addressed.

Pharmacodynamic properties:

- (a) The primary effects of medicine, with results in different animal species (ED50 values if possible), should be addressed.
- (b) Comparison of the effects of medicine with that of reference product(s), is valuable information.
- (c) Where relevant, the pharmacology of significant metabolites should be investigated.
- (d) The pharmacodynamic effects, especially those that might be of significance for adverse effects of medicine, should be studied and described.

Interaction studies, where relevant, should be included.

5.1.2 Pharmacokinetic properties:

- (a) To assist in the interpretation of toxicological studies, it is important to compare the exposure of the animals used in the toxicity testing with that anticipated in patients given the proposed therapeutic dose regimen data, which includes C_{max} (after a single dose and at steady state) and AUC.
- (b) MODULE 4 should, therefore, include comparative pharmacokinetic data for the parent substance and major/ active metabolite(s), where relevant, in human and all species used in toxicity, carcinogenicity and reproduction studies.
- (c) These data should preferably be obtained from the toxicity studies.
- (d) Other information (for example, $t_{1/2}$ and clearance) may be of value where important differences have been shown between animals and man.

5.1.3 Toxicology:

For medicines other than biological medicines the Toxicology of the medicine should be addressed.

- (a) A summary, or Expert Report, should be submitted for each animal species studied, with information on the sex of the animals, number of animals, dosage, route of administration, duration of study and toxic manifestations.
- (b) Important points to consider that pertain to pre-clinical toxicity, are:
 - Dose-response relationship
 - Time-response relationship

- Species specificity
 - Consistency of findings across studies with different species
 - Target organ specificity
 - Reversibility/ irreversibility of toxic effects.
- (c) Medicines that show specific toxicological effects, such as immunotoxicity, teratogenicity, nephrotoxicity, hepatotoxicity or neurotoxicity, should be investigated further, taking into account the points under b) above.
- (d) New medicines, which belong to classes that are known to produce a particular toxic effect, should be tested appropriately.
- (e) The possible mechanism(s) underlying the changes observed in toxicity studies need to be investigated and addressed.
- (f) Due to the local climatic conditions, the phototoxic potential of a medicine should be considered.
- (g) The points to address in the reproduction studies include fertility, embryonal toxicity, teratogenicity, peri- and post- natal effects.

5.2 CLINICAL STUDIES (eCTD MODULES 2.5, 2.7 & 5)

The following items should be addressed in conducting, and presenting the findings from, clinical studies:

- (a) The clinical data should be presented in a manner that allows easy cross-referencing to the index, other studies, and the professional information.
- (b) Data presented in support of the safety and efficacy of the medicine should be derived from clinical trials conducted in compliance with internationally accepted good clinical practice (GCP) guidelines. The studies should be properly designed and conducted and should be of acceptable statistical power (refer to the SA clinical trial guidelines), and any other ICH-approved clinical trial guidelines referenced therein). Where relevant, results published in peer reviewed scientific journals, should be submitted in full.
- (c) Clinical trials should be conducted with the formulation as applied for. Where studies have been

conducted with different formulations, comparative equivalence studies are required to enable extrapolation to the formulation intended for the market.

- (d) Individual patient data and case record forms from clinical trials need not be included in an application dossier (except in the case of bioequivalence studies where the individual plasma/ serum concentrations concentration/ time curves and derived pharmacokinetic data should be supplied). Tabulated individual patient data may be included in the application if the applicant considers it appropriate.
- (e) Studies designed to demonstrate the pharmacodynamic properties of a medicine should address the effect of the medicine, duration of effect, dose-response and tolerance. Additional action on e.g. the central nervous system, respiration, circulation, blood chemistry, liver and kidney function should be considered at the proposed therapeutic dose(s).
- (f) Pharmacokinetic studies should be conducted with the formulation as applied for. All relevant pharmacokinetic data shall be given, such as amount and rate of absorption after various routes of administration, plasma concentration, half-lives, drug clearance, drug metabolism, handling by transporters, as well as the routes and rates of excretion. Data on major/ active metabolite(s) should be included if applicable.
- (g) The pharmacokinetic studies should be carried out with both single dose and multiple doses to steady state within the recommended dosage range. Where applicable the plasma concentration(s) producing pharmacological and/ or therapeutic effects as well as adverse effects, should be presented. Possible dose and time-dependent pharmacokinetic effects should be addressed. The consequences of genetic polymorphism on transporters and metabolic enzymes should be discussed, if applicable.
- (h) The trial design of the relevant clinical studies should be such that the safety and efficacy of the medicine can be established in comparison to either a placebo and/ or a medicine registered by an RRA.
- (i) The description of the studies should include patient population size and diagnosis, in- and exclusion criteria, test and comparator/ reference medicine dosage regimens and duration of therapy, parameters assessed for efficacy and safety, including results of special investigations.
- (j) It should be noted that the randomised, double blind, placebo and/ or active controlled trial design

remains the gold standard for establishing the efficacy and safety of medicines.

- (k) Detailed statistical results should be presented. The analysis conducted should be consistent with that pre-specified in the protocol and statistical analysis plan (SAP). The SAP must be finalised before unblinded trial data are available.
- (l) The dosage of the active comparator/ reference should be in line with that approved for the specific therapeutic indication.
- (m) The patient dropouts should be addressed, including the time of and reason(s) for withdrawal/ drop-out.
- (n) To enable evaluation of safety of the medicine it should be noted that the long-term safety, particularly for medicines proposed for chronic use, or where repeated treatment courses are to be used, should be addressed.
- (o) Whilst the medicine is in the evaluation process Applicants should notify the Authority of any prohibition or restriction imposed by the competent authorities of any country in which the medicine is marketed and of any other new information which might influence the evaluation of the benefits and risks of the medicine concerned.

6. DOCUMENT/ DATA REQUIREMENTS FOR NEW REGISTRATION

This section outlines the new-registration document/ data requirements for the various evaluation pathways. Clinical document/ data requirements for variation applications can be found in the “SAHPGL-HPA-06, Variations Addendum for Human and Veterinary Medicines”.

6.1 FULL REVIEW REQUIREMENTS

6.1.1 Applicants cover letter (M1.0)

6.1.2 Proposed PI and PIL (M1.3)

6.1.3 Standard references (if applicable) (M1.3)

6.1.4 Administrative and Clinical technical screening checklists (M1.2.5)

6.1.4.1 Registration status and dates of approval with other regulatory authorities (M1.10)
[Applicants are requested to highlight SAHPRA's RRAs on this list]

6.1.4.2 Risk Management Plan (RMP) refer to RMP guideline (see SAHPGL-CEM-PV) (M1.8.2)

6.1.4.3 Latest Periodic Safety Update Report (PSUR)/ Periodic Benefit-Risk Evaluation Report (PBRER) if already registered by a RRA, if applicable – (M5)

6.1.4.4 The latest approved PI and PIL with SAHPRA date of approval (M1.3), applicable for Variations applications.

Pre-clinical data (proof of concept, in vitro/ in vivo data, animal data)

6.1.5 Overview of pre-clinical data (M2.4)

6.1.6 Synopsis of pre-clinical findings of relevance to humans (M2.6)

6.1.7 Pre-clinical data expert report from the applicant (M2.4)

6.1.8 Full pre-clinical data (M4)

6.1.9 The studies include pharmacological properties, pharmacokinetic, and toxicology, this information should be integrated across studies and across species, and exposure in the test animals should be related to exposure in humans given the maximum intended doses (M4)

Clinical data

6.1.10 Overview of clinical data (incl. safety, efficacy, pharmacology and benefit/ risk analysis) (M2.5)

6.1.11 Clinical expert reports on safety and efficacy from the applicant (M2.5)

6.1.12 Synopsis of each clinical study included in the application (M2.7)

6.1.13 Full clinical study data with formulation as applied for (FAAF) (M5)

6.1.14 Studies on the molecule/ medicine demonstrating its pharmacology including mechanism of action (if applicable or possible) and the pharmacotoxicology.

6.1.15 Studies demonstrating pharmacodynamic properties (M5)

6.1.16 Studies demonstrating pharmacokinetic properties, including PK/PD relationship, and where relevant, pharmacokinetic properties in special populations (e.g. hepatic, renal, gender, race,

elderly, children, other age groups) and pharmacodynamic/ pharmacokinetic interactions with other medicines relevant to the indication and target population (M5)

6.2 ABRIDGED REVIEW REQUIREMENTS

Some requirements may not be applicable to a certain application type for abridged review:

- 6.2.1 Full review requirements 6.1.1 – 6.1.16
- 6.2.2 Un-redacted rapporteur assessment reports from RRAs, if available (M1.10)
- 6.2.3 The relevant reference PI approved by a RRA M1.3.5 (eCTD spec 3.1)
- 6.2.4 Declaration that the information in the application is materially the same as the information submitted to the regulatory authority (name the RRA) which approved the medicine (include approval date) (M1.10.4.2)
- 6.2.5 Correspondence between the Applicant and other reference RRAs, concerning queries relating to safety, efficacy, risk/ benefit and RMP issues (if not included in the un-redacted assessment report). Detailed explanation/ reasons if registration/ approval was refused by a Regulator with which SAHPRA aligns itself (M1.10)
- 6.2.6 The latest approved PI and PIL with SAHPRA date of approval (M1.3), applicable for Variations applications.

6.3 VERIFIED REVIEW REQUIREMENTS

Some requirements may not be applicable to a certain application type for verified review:

- 6.3.1 Full review requirements 6.1.1 – 6.1.16
- 6.3.2 Full review requirement 4.1.6, if/ when applicable for specified molecules and indications
- 6.3.3 The relevant, primary reference innovator PI approved by SAHPRA (M1.3)
- 6.3.4 The relevant secondary reference PI approved by RRA, if applicable in instances where the local innovator PI is materially outdated (M1.3)
- 6.3.5 The latest approved PI and PIL with SAHPRA date of approval (M1.3), applicable for
- 6.3.6 Variations applications.

7. REFERENCES

The following related documents are referenced:

- 7.1 General Information Guideline (SAHPGL-HPA-07)
- 7.2 Variations Addendum for Human and Veterinary Medicines (SAHPGL-HPA-06)
- 7.3 Guideline on Professional Information for Human Medicines (Categories A and D) [SAHPGL-CEM-02]
- 7.4 Guideline for Patient Information Leaflet for Human Medicines (Categories A and D) [SAHPGL-CEM-03]
- 7.5 Biosimilar Medicines Guideline (SAHPGL-PEM-BIO-02)
- 7.6 EMA Guideline on clinical development of fixed combination medicinal products (except for HIV, Malaria and TB medicines where the SAHPRA guideline applies) [EMA/CHMP/158268/2017]
- 7.7 SAHPRA Fixed Drug Combination Guidelines for HIV, TB and Malaria (SAHPGL-CEM-PRE-02)
- 7.8 EU variations classification guidelines (see EMA website: <https://www.ema.europa.eu/en/homepage>)
- 7.9 Quality & Bioequivalence guideline (SAHPGL-PEM-02)
- 7.10 SAHPRA eCTD Specification 3.1 and Guidance for Module 1 and Regional Information (SAHPGL-HPA-09)
- 7.11 Medicines and Related Substances Act, (Act 101 of 1965), as amended (Government Notice No. 859, Government Gazette No. 41064, 25 August 2017)
- 7.12 General Regulations (Medicines and Related Substances Act, 1965) 25 August 2017
- 7.13 ICH Guidelines (S, E and M series) (see ICH website: <https://www.ich.org/>)
- 7.14 EMA guidance on a new therapeutic indication for a well-established substance (see EMA website: <https://www.ema.europa.eu/en/homepage>)
- 7.15 EMA guideline on the categorisation of extension applications (EA) versus variations applications (V) (see EMA website: <https://www.ema.europa.eu/en/homepage>)

- 7.16 General Guidance document on Quality, Safety and Efficacy requirements for Biological Medicines (SAHPGL-PEM-BIO-06)
- 7.17 SAHPRA Guideline for submission of Extension applications (SAHPGL-CEM-PRE-03)
- 7.18 SAHPRA reliance guideline (SAHPGL-BAU-01)
- 7.19 SAHPRA clinical trial guidelines (see SAHPRA website <https://www.sahpra.org.za/guidelines/>)
- 7.20 SAHPRA RMP guideline (SAHPGL-CEM-PV-03)
- 7.21 Guideline for Patient Information Leaflet for Human Medicines (Categories A and D) [SAHPGL-CEM-03]

8. VALIDITY

This guideline is valid for a period of 5 years from the effective date of revision and replaces the previous revision of this guideline (i.e. v3). It will be reviewed on this timeframe or as and when required.